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Reflections on Research and Clinical Practice

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Thinking postmodern and practising in the enlightenment: Managing uncertainty in the treatment of children and adolescents

Bernadette Wren
Tavistock and Portman NHS Foundation Trust, UK

Abstract
In this paper, I explore how postmodern ideas about gender may impact on clinicians working with gender variant children and adolescents. The postmodern turn has built on the feminist rejection of the idea of 'essential' gender, to further interrogate accepted conceptions of sex and gender and the stability of all identity categories. Some queer theorists have taken a further step, viewing all gender as fictional and artificial and celebrating the subversive potential of transgender identities. However, those working clinically with trans adolescents may experience a troubling tension between, on the one hand, a view of sex and gender categorisations as undecidable and fragmented (as postmodern theory suggests), and, on the other hand, the apparent need of many for a coherent and settled sense of self. In particular, how do we justify supporting trans youngsters to move towards treatment involving irreversible physical change, while ascribing to a highly tentative and provisional account of how we come to identify and live as gendered? I conclude that the meaning of trans rests on no demonstrable foundational truths but is constantly being shaped and re-shaped in our social world. Clinicians must be accountable in this process; far from succumbing to a paralysing relativism, the task for clinicians is to be highly attuned to our young clients' complex narratives and to question our complex investments in the positions we adopt.

Keywords
Gender, feminism, transgender, adolescence, postmodernism

Corresponding author:
Bernadette Wren, Tavistock and Portman NHS Foundation Trust, Tavistock Centre, 120 Belsize Lane, London NW3 5BA, UK.
Email: bwren@tavi-port.nhs.uk

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Introduction

This is a paper about explanatory narratives in the field of atypical gender identity development and about the broader meta-narratives that sustain them. In particular, it is concerned with the impact on clinicians in the field of gender identity of the increasing pervasiveness of 'postmodern' ideas about gender, the body and the self.

I write as a clinician attached to a state-funded (National Health Service) clinic for children and adolescents whose experience of the body is at odds with their gendered self. Our clients are offered an opportunity to think about any problematic impacts of their atypical identification – on their emotional well-being, their relationships, their health, education and so on. We also serve effectively as gatekeepers to their accessing hormonal intervention.[1] In the individual and family psychotherapy offered at the clinic, a very open exploration of possibilities is advocated, promoting the idea of tolerating and living with conflict and contradiction. However, working with these gender dysphoric children and young people, the non-trans clinician (especially operating in the discursive context of a public mental health service) may sometimes struggle to fully grasp the determination of otherwise rational and reflective young people to define themselves irreversibly with the help of powerful chemicals and, ultimately, surgery.

The question I want to address here is about the ways of understanding about gender development that we might count on to ground our practice of treating these young transgender people with physical interventions. To this cis-gendered clinician, certain questions recur: Can 'postmodern' ideas about the non-fixity and instability of gender serve the perplexed clinician? Can we forego the grounding of our ideas in demonstrable certainties? Operating with a postmodern notion of gender, can the clinician justify irreversible physical intervention? Or are we – adapting a phrase from Harris (2004: 1083) – thinking postmodern but practising in the enlightenment?

Postmodernism

Many of us who are influenced in our clinical work by systemic thinking and ideas from relational psychoanalysis have come to value aspects of the postmodern turn in psychotherapy. Postmodernism – a loose alliance of intellectual perspectives – has been a turn towards fragmentation and multiplicity. As a feminist, I have been aware of the concern of some feminist authors (e.g. Burman, 1992; Gill, 1995) that postmodern notions of gender, that regard subject positions as multiple and fragmented, risk offering no clear path to political action and may deny the possibility of an identity around which women can collectively mobilise. At the same time, as a therapist, I have questioned the relevance of the appropriation of postmodernism to mental health and psychotherapy, where meaning and coherence are still valued as goals of the work. Nevertheless, I believe it is the case that important social, political and ethical issues in psychotherapy are being addressed with urgency in the debates on the meaning and value of the postmodern turn. In these debates, the fundamental modernist notions that underpin psychology and the social sciences

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have been questioned: the stable, coherent, knowable self; rationality as the highest form of mental functioning; science as the paradigm for any and all socially useful forms of knowledge (e.g. Flax, 1996).

Postmodernism – in its poststructuralist version – represents in the social sciences a reaction to structuralist claims that it is possible to discern and describe with scientific rigour the underlying (causal) structures that systematically produce the meaning of any cultural formation (Harvey, 1990; Rorty, 1995). It calls on us to examine the assumptions that produce and reproduce what are intended to be fixed categories of meaning (Derrida, 1973). It argues that we cannot properly speak of objective and universal truths because it recognises that we can never stand outside the conceptual frameworks we are trying to explain. Within the postmodern movement, deconstruction has been a particular kind of critical response to beliefs in absolute essences and oppositions. A deconstructionist analysis, chiefly associated with the philosopher Derrida (1967/1976), would point to the way we seem to be locked into forms of talking that imply relations of dominance, of one-way dependency. Following Derrida, postmodernist feminists take a stand against the 'violence of certainty' (Hepburn, 1999). They reject the modernist desire for certain legitimating meta-narratives; those, for example, which treat the anatomical distinction between the sexes as determining psychological development with respect both to character and desire, and which serve to marginalise women and other minorities. Such meta-narratives, writes Goldner, produce subjects by 'separating, classifying, ranking and evaluating persons in hierarchies of normality and morality that are . . . hopelessly entangled' (Goldner, 2011: 160). For Butler (1990), the seemingly foundational meta-narratives of the male–female and hetero–homosexual binaries uphold each other, each asserting its own form of regulatory power.

The deconstructionist view especially highlights the inherent instability of the terms in a binary (the binary being a particularly pervasive way of structuring experience in Western thought), as well as enabling an analysis of the ways in which the terms and the relation between them have developed across time. The idea – highly pertinent to gender studies – is that binary identity categories, far from being dichotomous, are best seen as co-defining (Rorty, 1995). One term lends intelligibility to the other with each taking on meaning in a context of power inequalities. For example, homosexuality is not 'opposed' to heterosexuality, but internal to it and defining of it (Wiseman and Davidson, 2011). Femininity is not opposed to masculinity, nor black to white; they are pairs in a hierarchical relation and the valued member of the pair characterises itself primarily by denying unwanted characteristics of the other. Constructs usually regarded as closed are pervasively organised by what they define as external to them. Any attempt to fix meaning is understood to be subverted by ambiguities within the object of study and within the interpreter. (It is probable that the apparent 'oppositions' between structuralism and poststructuralism, modernism and postmodernism also need to be understood as terms whose meanings are inter-related and co-defined in complex ways.)

A poststructuralist concern with gender and sexuality uses deconstructionist critical approaches to question our accepted conceptions of sex and gender and

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to explore the limitations of traditional identity politics. But queer theorists have gone further, questioning the very idea of a 'real' material sex underlying our social constructions, and viewing transgender and transsexual people as inevitably transgressing conventional boundaries, and thereby helping to dismantle binary oppositions such as masculinity-femininity and nature-nurture (Butler, 1990; Sedgwick, 1993). In this paper, I explore what seems at times to be a premature, perhaps naïve, celebration of the subversive potential of trans. People with clinical responsibilities may experience a troubling tension between what influential postmodern theory suggests about the undecidability and fragmentation in sex and gender categorisations, and the need of trans people for a coherent and settled sense of self.

Postmodernism in clinical work

Postmodern ideas have found their way into the work of clinicians whose thinking has been informed by a socio-political ideal of openness to alterity or otherness. For Lyotard, postmodern knowledge 'refines our sensitivity to differences and reinforces our ability to tolerate the incommensurable' (Lyotard, 1979). The epistemological role of the 'Other', the marginalised, has been seized on as offering critical perspectives on reality and offering larger insights into how society functions. The alienated Other challenges the projections of self by those in power. Hence, poststructuralism has been of interest to clinicians who accept that all clinical theory and practice are pervaded by strongly entrenched categories of race, ethnicity, class, gender, sexuality: the belief that, as therapists and as clients, we organise our experiences, and the data of other people's experience, in specific ways to give them coherence and narrative meaning, and to sustain personal and socio-cultural meaning systems that are important to our self-definitions (Fairfield et al., 2002). All events reported in therapy are held to be experienced, narrated, interpreted and retold from particular perspectives and in specific contexts that carry social, emotional, psychological, political and historical meaning. It is implied that all aspects of the clinician's participation represent just one among many possible ways of organising experience. Hence, the therapist must maintain a position of uncertainty in relation to her own convictions, once these are seen as inevitably a contingent, local product of her (personal and professional) positioning at a particular socio-political moment. Client and therapist come to accept that there may be multiple plausible accounts of what is happening for the client. 'It now sometimes appears that the capacity to contain the dread of not knowing is a measure of clinical virtue; the fewer the convictions, the braver and the better' (Mitchell, 1993: 43).

Attachment, dependency, boundaries – these may still be our key constructs in clinical work, but gender, race, class, sexuality and their intersections are not subsidiary concepts, but are inextricably woven with them (Fairfield et al., 2002). From this perspective, treatment is likely to aim towards helping clients both to bear uncertainty and to help develop the capacity for critical reflection on how their reality is constructed both historically and in the moment. The aim – whether from a psychoanalytic (Goldner, 2002) or a narrative (White and Epston, 2007)

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perspective – is to enrich and develop the patient's story. A particular interpretation may be useful and will be judged on what it makes possible in the patient's life and relationships. This is a step on from a hermeneutic position in which 'truth' is subsumed to 'narrative truth' in therapy where realities are co-constructed (Schafer, 1994). In poststructuralism, what matters is not just dialogic co-creation of meaning (drawing self-critically on a range of traditions), but the 'discursive formations and cultural narratives that set the terms for what the dialogic partners can think and know' (Goldner, 2002: 160).

A Lacanian perspective goes further: the postmodern therapist helps the patient learn, as Flax (1996) writes, 'to tolerate the absence of meaning, the limits of narrative organisation and the ineradicable persistence of intelligibility' (Flax, 1996: 589). For instance, if memories are always reconstructions, we will often not know much about literal events and will accept that much of what drives us is unconscious. In the face of this insufficiency of language, all of us, patients and therapists, must weather the psychic turmoil of not constructing a coherent narrative. It debunks the idea that simply providing another narrative will move people on (Frosh, 1995). Multiple narratives are possible precisely because all language falls short; and because there may be something unsymbolisable at the heart of human experience, something that cannot be mastered through language. Analogically, we see Hayles (like Lyotard) identifying 'scientific chaos' as itself 'a postmodernism':

> Many scientists have commented that working on chaos has allowed them to renew their sense of wonder. Although they do not put it this way, they intimate that chaos has given them a sense of being in touch with the Lacanian real. For them, chaos is an image of what can be touched but not grasped, felt but not seen . . . chaos presents them with a resistance that alleviates the fear of mastery. (Hayles, 1990: 292–293)

Taking this position to represent an alternative ontology would risk disabling the therapist who cannot easily give up the responsibility to help the client create some order in his or her narrative. But if we view postmodernism as a willingness to take a seriously critical review of the project of modernity, then the postmodern clinician's relationship to conventional clinical perspectives is not to disavow them entirely but to put them relentlessly in question, to challenge the solidity and coherence of their founding assumptions, theoretical models and their methods of generating evidence.[2]

Postmodern approaches to gender

Work on gender has been central to the postmodern debate in therapy. The postmodern way is to see gender not as monolithic and linear, but as a creative compromise in endless negotiation with the self, other and culture. Clinicians practicing and theorising in more traditional ways have been criticised for seeming to enforce particular views of normality. Corbett (2009), a psychoanalyst writing about feminine boys, critically notes Chassageut-Smirgel's warning that a too close

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mother–son bond, too much access to femininity, can lead to 'psychic death' (Chassageut-Smirgel, 1976: 349). And Corbett characterises the preferred therapeutic interventions of Zucker and coworkers (e.g. Zucker and Bradley, 1995), that seem to express similar unquestioned presuppositions about gender, as 'modes of care that do not proceed from an adequate understanding of the range that is masculinity, and the mutability that is gender' (Corbett, 2009: 354).

Arguments supporting the deconstruction of conventional gender categories and critiquing the idea of core or monolithic gender identity have been advanced by a number of feminist writers utilising a broadly postmodern frame. The idea that gender is mutable has been inflected, not exclusively, as malleable and evolving over time (Goldner, 1991, 2002; Layton, 1998); as so multi-layered as to be best understood within a framework of complexity or chaos theory (Harris, 2005, 2004); and as involving the agency of the individual, as well as prenatal and parental forces (Benjamin, 1988). These conceptualisations build on challenges to essentialist definitions of femininity and assumptions of a universal female identity, and emphasise the discursive power and the ambiguity of gender. Even the word 'feminist', if it is taken to require the assumption of a unitary and fixed female identity, can be misinterpreted as insensitive to a notion of gender as unstable and fragmented, and to the potential oppressions inherent in all gender roles (Hepburn, 1999).

These approaches, although varying in many crucial respects, de-pathologise homosexuality and various other non-normative practices, and question the way that psychological and psychotherapeutic disciplines have defined what counts as deviant when it comes to sexuality and gender identity. The broadly postmodern case is that while it may be that all children in a culture must come to terms with gender difference at an early stage, still children's understanding of their own sexed bodies and of gender are always complexly determined, individual processes. Postmodernists give up the idea of any 'inborn' masculinity or femininity, and recognise instead the unfolding of a complex set of exchanges between self and others in which gender constructs can take on a rich variety of meanings and gender performances are put to a wide range of uses. Drawing on chaos and soft systems theories to address the repeated tension between regularity of pattern and variation in the emergence of the gendered self, Harris writes,

> with a developmental theory thus attuned to multiplicity and variation, what comes to light in clinical work are the vicissitudes of gender experience, with gender identity as a frame for doing various psychic and relational tasks in certain ways. (Harris, 2005)

Harris advocates seeing gender as function, not structure, arguing that it can be seen as a 'good enough compromise of our needs, historical conditions, and life circumstances' (Harris, 2005). Indeed, it is well for the clinician to consider that often we can only understand the simple in terms of its origins in the more complex (cf. Byrne, 1998). But it means that the clinician cannot claim to always know what is best. The therapist needs to be prepared to accept ambiguities and

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indeterminateness in the individual's gendered experience, to be open to the idea of a gender identity serving many diverse psychological needs and functions.

> Coherence, including gender coherence, is one of those "necessary fictions" in psychic life and often a requirement of our theories. From various sources – cultural, psychoanalytic and personal – tolerance for and interest in incoherence is on the rise. (Harris, 2005: 164)

These feminist postmodern authors have, in different ways, interrogated social, psychological, political and biological understandings of gender, while playing with ways of incorporating these understandings into a viable model of gendered subjectivity. Theorists taking a queer perspective tend to take a less compromising view, idealising trans as a transgressive force (e.g. Bornstein, 1994) and insisting on the cultural construction of the material body to the point of attacking any notion of gender that makes 'literal' reference to a body as a knowable prediscursive entity (e.g. Butler, 1990; Sedgwick, 1993). Prosser challenges this implied syllogism – 'transgender is queer is subversive' (Prosser, 1998: 29) and he sets out to address the ways in which 'material flesh may resist its cultural transcription' (Prosser, 1998: 7).

Postmodernism and psychotherapy

Now it is certainly true that psychotherapy – an inherently sense-making enterprise – can only take on a very partial version of postmodernism (Frosh, 1995). Nevertheless, for clinicians, these borrowings from poststructuralism and chaos theory spread influential new perspectives on gender and the therapeutic project with gender dysphoric people. They encourage clinicians to move away from simplistic binary ideas about gender identity as potentially stable and monolithic – ideas which fail to accommodate the variation in gendered experience which we see at a clinic for transgendered youth. And this leads to the question of *how* we find ways to incorporate into our clinical work with transgender children and adolescents these ideas about gender multiplicity and the indeterminate, (perhaps endlessly) negotiated nature of gender identity development. The young people coming to the service where I work assert passionately uncompromising narratives about their self-identity as male or female, despite the conflicting 'evidence' of their sexed bodies. And for the clinician, charged with the responsibility of accepting a young teenager onto a physical treatment programme, it is especially tempting to want some grounding in certainty to justify one's support of a client in gaining access to hormonal or surgical interventions. But where might this certainty be sought?

A postmodern clinical perspective on ways of knowing about transgender identity development

I have identified four kinds of grounding that are called on to bolster the decision to support physical intervention for young trans people. As I see it from a

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postmodern perspective, each grounding fails to offer the clinician especially strong or unequivocal evidential support for physical intervention. One kind of grounding is based in the confidence of the young people that their own subjective experience is the best guide to making irrevocable decisions about treatment. Three kinds of grounding have a basis in empirical research work, versions of which have found their way into the public discourse about transgender and may be drawn on by young people and their parents who are often avid consumers of internet-based sources of information. Of course, not all strands of research (in whatever form they enter the into gender identity debate) chime well with the young clients' preferred self-narratives: indeed, research may be a reference point for our clients only in the act of rejecting it.

Privileged access

In my clinical work and research with adolescents with atypical gender identity development (Wren, 2000a), I have been very struck by their commitment to living and being embodied as the sex they feel themselves to be – a position typically, but not always (Wren, 2004), held with great conviction and certainty. They talk about knowing the truth of their gender identity in terms I describe as 'privileged access'. This is the notion that transgender people are the ultimate authority on their gender and their relationship to the body. Their certainty, they imply, is its own guarantee and cannot be questioned by others. Self-deception is not possible; no one else can mount a contradiction.

It is worth exploring what kind of *knowing* this is: it privileges the subjectivity of the trans person and is often seen as arising from direct bodily experience – an experience of non-identification with the sexed body as it is. Prosser (1998) puts the profound sense of bodily discomfort, or bodily unbelonging, at the heart of transgendered narratives. Indeed, this discomfort can be regarded as the 'reality' to which postmodern theoretical accounts fail to do justice. As Connell (2009) notes, poststructuralist theory has often exaggerated the docility of bodies. Prosser draws on the work of psychoanalyst Anzieu who suggests that a sense of self derives from the experience of the material skin, and who insists on the supremacy of touch over sight in the development of the body image. This allows Prosser to argue that a feeling of bodily integrity is crucial to successful functioning (Prosser, 1998), and he implies that this sense of identity and continuity of the self forms an essential part of self-belief and self-certainty. And yet, others have argued (e.g. Stryker, 2008) that Prosser does seem to offer a conception of self (or at least bodily self) which is implausibly independent of cultural demands. Postmodernism requires precisely a cultural analysis of a phenomenon, even when that phenomenon is the seemingly direct experience of embodiment.

When our young clients give an account of a childhood of feeling differently gendered, and report a prolonged and intense experience of the wrongness of the body, their knowing can hardly be understood as a kind of *evidential* knowing. It is not a question of giving grounds for the belief (Wren, 2000b). Wittgenstein (1969) has helped us to see that talk of how we feel 'inside' does not have meaning in

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virtue of standing for objects with which we become acquainted through internal inspection. In introspecting, they may be sure of their gender identity, but not by 'verifying' their view. Indeed, from a poststructuralist position, talking about one's inner life does not actually describe anything. A postmodern fragmented self can experience the present vividly; indeed, we can say that immediacy of events is the stuff of which consciousness is forged (Harvey, 1990). Talk about our feelings, our identity is communicative work to achieve certain purposes in the world (Rorty, 1991). Out of elements available in the culture, such talk produces a version of subjectivity and seeks to establish it through one or more of the culture's legitimating discourses (Foucault, 1970/1996). Accordingly, the young people's story can be read as just that – a story, one among many, with no privileged access to the truth. Indeed, their claims would be seen as claims to identity, moves in a dialogic game where they wish their agenda to dominate – not more or less accurate descriptions of their inner world. As Rorty puts it, our multiple vocabularies are 'tools rather than mirrors' (Rorty, 1991: 158). Even when trans people tell a convincing story of early years of gender dysphoria, these can be seen as belonging in a recognised tradition of legitimising stories to convince a prejudiced world of the integrity of their claims, statements claiming an identity, aiming to persuade others of one's status and saying something about how one wants to relate to people. (A number of writers have pointed to the way that transsexual people seeking surgery have conformed their story to what the conventions of the autobiographical narrative required within medical circles, e.g. Prosser, 1999). But, given such a reading, the clinician may feel that she cannot steer her own practice solely on the basis of the young client's self-narrative, when that very narrative may be what is up for re-examination and re-telling in the therapy room. This is especially so in a service for under-18s since, as Roen (2011a) notes, 'both gender and adolescence are characterised by fluidity, changeability and uncertainty' (Roen, 2011a: 64).

Neurobiological explanations

There has been a move to ground claims to transsexual status in some form of neurobiological evidence. Many young transgender people bolster their identity claims with a belief that such evidence will soon be available to show they literally have 'male brains in female bodies', or vice versa. Some professionals similarly place centrally in their work the hope that at some point physiological evidence will justify the treatment offered. This scientific work is in its infancy (e.g. Van de Beek et al., 2009; Zhou et al., 1995) but much faith is placed in any innovative discoveries in the field (Bao and Swaab, 2011, provide a good overview of research on the sexual differentiation of the human brain). Fausto-Sterling has been a leading critic of simplistic views of the way biology influences gender development. She has recently criticised what she calls the 'agnostic' position of both Zucker et al. (2012) and De Vries and Cohen-Kettenis (2012) for failing to properly link biology (genes, hormones, temperament, brain structure, etc.) and the other psychosocial, cognitive or psychodynamic factors and processes they deem somehow relevant in the evolution of gender identity. She reminds us that

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> brain development itself, especially in children, is to a large extent directed by social and sensory experiences; so too is hormone biology and physiological regulation . . . a well-developed theory of gender identity needs to place this dynamic at its centre. (Fausto-Sterling, 2012: 400)

While we can acknowledge how the turn to biology has been an attempt to shift from a stigmatising psychological approach to transsexuality to a seemingly less pathologising account, a postmodern view would insist that the body is always already a discursive entity, so that biology and physiology are only determinative at the most extreme limits. The body cannot be 'known' outside of, or apart from, the grids of intelligibility that exist in our culture, any more than the mind can. And our meaning grids are steeped in conventional gender ideas. This critique can also be directed more widely at current trends in the field of mental health where the key question of who decides what knowledge is (and who knows what needs to be decided) is increasingly directed beyond social sciences to the modernist bastion of natural sciences (cf. Lyotard, 1979). Feminist author Hausman has articulated her discomfort that the 'medical' model of transsexuality gives the power of definition and judgement too readily into the hands of the medical establishment keen to define and regulate gender (Hausman, 1995). More (1999) comments on the political dimension of all knowledge claims: theories that at one time prove liberating and fruitful may at another time prove a liability.

> As Blechner argues of homosexuality, Freud's shift from biology to psychology was perceived as liberating and yet when LeVay and others moved it back that was also liberating, because the oppressive hegemony had moved, as it always will, to crush new ground. This is the problem of having one causality. (More, 1999: 251)

If, in postmodern terms, causal theories are read inevitably as products of the social nexus in which they were developed, they will always be constraining in particular ways; a measure of freedom is (temporarily) gained by looking for other versions of causality.

Additionally, this grounding model, while undermining conventional (pathologising) psychological accounts, works to biologise – and thereby limit and oversimplify – ideas and concepts of sex and gender. As we have seen earlier, Butler (1990) dismissed the claim that feminism needs to have any kind of grounding in materiality. For Butler, the desire for biological determinism is

> a kind of fabulous release from the anxieties and ambivalences that go with being sexed . . . to accept it as an explanation is to sacrifice everything that is dynamic and important about the categorisation of sex and its possibility of being revised and reworked and re-lived. (Butler in More, 1999)

From this perspective, a move towards bodily alteration, predicated on a causal account grounded in biology, can appear as a premature closure on the rich and creative possibilities of self-invention in gendered life. As indicated earlier, a tense

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debate developed between some feminists who see the trans person's desire to 'fix' their gender through hormonal and surgical techniques as a perpetuation of sexist norms (e.g. Hausman, 1995; Raymond, 1979/1994), and those who recognise the integrity of many trans people's insistence on the absolute need for a settled sense of self, with gender fixity as a prerequisite. In 2004, Butler returned to this debate with an acknowledgment that various kinds and degrees of stability are required to sustain a liveable life (Butler, 2004).

Normative developmental accounts

Some therapists in the transgender field take as their basic grounding belief the idea that the phenomenon of cross-gender identity points to a troubled early development. This view may be grounded in the clinical and research work that has focused on psychopathology in gender dysphoric children and their families, looking, for instance, at maternal mental illness, 'failed' or absent role models and the perverse encouragement by parents of inappropriate gendered behaviour (e.g. Zucker and Bradley, 1995). On this view, at its most simplistic, all children in a culture are basically travelling the same path in coming to terms with gender difference and should ideally reach the same goal – a settled, consistent, core gender identity, at one with the sexed body. An approach to gender identity grounded in such a normative, teleological developmental account implies a conviction that the certainty expressed by trans young people is evidence of a developmental crisis. A clinician committed to an account of development-gone-awry is likely to see gender categorisations as static and binary, and physical intervention as misguided, representing a 'failure' in treatment, the surrendering to a delusion.

From a critical postmodern perspective, there are a number of problematic issues here. The turn towards a family story line to explain atypical gender identity development is itself vulnerable to the (feminist) criticism that it relies on and reinforces the politics of traditional family values and gender stereotyping, omitting any concern for the socio-political dimension of gendered development. It also implies that the aetiology of gender identity development is already mapped, although in fact our knowledge of the key processes and structures, and the relationships between them, remains largely hypothetical. Additionally, and following Coates (1997), we have to be very careful not to conflate developmental lines with the sequential phenomena of physical maturation. Once we posit a linear sequence of development, Coates insists, we make choices about normality and deviance, and find ourselves imposing fixed notions of health, normality and disease in the arenas of sex and gender. If postmodern clinicians do not entirely dismiss developmental accounts, they must critique premature and simplistic hypotheses about processes that promise to be profoundly complex.

Certain developmental accounts do offer a richer, more multi-layered, and less obviously ideological, model of the early origins of gender identifications. Across several research streams into infant behaviour and cognition, it is becoming clearer that most functions emerge where intrapsychic, interpersonal and cultural processes dynamically intersect. At a *macro* level, it may appear as if a few key unitary

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developmental clusters (like male and female gender identity) slowly and smoothly appear, as prewired developmental achievements. Nevertheless, at a more fine-grained *micro* level, what is striking is the non-linear, largely unpredictable and individually crafted nature of the emergence of many biological and psychological processes. Current models of the emergence of a conscious awareness of gender are more likely to draw on a dynamic embodied systems framework, as outlined by Harris (2005) and Corbett (2009), whereby behaviours and cognitions arise from the interaction of a number of contributing systems. A person's gender identity, as Harris (2005) speculates, is idiosyncratically 'softly assembled'. Her understanding of biological and psychosocial development emphasises the non-linear course of developmental change and places a premium on the self-organising properties of experience and the 'features of asynchrony, gaps, ruptures and novel integrations' (Harris, 2005). Fausto-Stirling (2012) calls for longitudinal studies of the crucial presymbolic dyadic processes through which, the evidence suggests, gender becomes embodied in infancy – these in turn leading to symbolic gender representation and the concomitant internalisation of identity.

The thorough-going postmodernist clinician holds herself somewhat ambivalently in relation to these explanatory models. She will necessarily be suspicious, although not necessarily dismissive, of any reliance on the explanatory power of developmental narratives. This is partly an ethical issue: a wish to restore some dignity to those whose transgender identification feels to them viable, respectable and worthy of value, by avoiding causal models which seem to point lazily to a failed or damaged developmental end-point. A posture of informed suspicion also highlights the way that developmental models tend to serve a performative as well as a descriptive function; as we have noted, it is precisely this *constitutive* function of gender norms which is questioned within a postmodern frame.

In my experience, most trans young people are not interested in the possibility of a complex, more or less speculative, developmental account of their gender identity – unless this unequivocally underpins the legitimacy of their wish for, and entitlement to, help to physically transition. Many will acknowledge the impact of early experiences or particular relational patterns (traumatic or otherwise) on their sense of self – but not on their *gendered* identity. This is partly because the hypothesised events in early infancy are beyond recovery into memory and thus seem unreal and fantastic; partly because from their marginalised and threatened subject position, explanatory narratives are too often heard as blaming stories, with children cast as victims of parental perversity or ineptitude; partly because the young people may prefer to assert their gender identity as a 'given', not something to be unpacked and potentially re-assembled. But the lack of interest in complex causality is also testament to the way in which developmental accounts are typically put to use – to bolster the usual binaries in mental health: normal/abnormal, straight/perverse, healthy/sick. Young trans people may believe that such accounts can only prejudice their chance of having their experience of the body taken seriously as an intelligible identity, lived with dignity and integrity.

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Psychological adjustment

Clinicians may find another kind of grounding in empirical research that focuses on psychological assessments of young gender dysphoric and trans people. For instance, there is evidence that on standardised tests of psychological functioning and well-being, trans adolescents as a group do not score in the clinical range at the time of application for sex reassignment surgery (Smith et al., 2001; Wallien, Swaab and Cohen-Kettenis, 2008). Other studies have shown, however, that young people with gender identity problems show more frequent internalising problems (anxiety, social withdrawal or depression) than non-referred children (Cohen-Kettenis et al., 2003; Cohen-Kettenis and van Goozen, 2002; Di Ceglie et al., 2002; Zucker and Bradley, 1995). Meanwhile, guidelines are emerging from studies in the Netherlands about which young people, assessed psychometrically, are most likely to do well from early intervention (Smith et al., 2001). Their evidence suggests that an absence of serious psychosocial problems, the presence of a supportive family and a long, consistent history of pronounced gender dysphoria point towards a likely good adjustment to physical intervention (Cohen-Kettenis et al., 2008).

These latter kinds of studies are useful in providing support for the practices of the more liberal gender identity clinics. But such findings are of course vulnerable to the postmodern critiques of modernist science once we begin to examine and question the measures used and the assumptions behind them, the contexts and sites of testing, the investment in particular outcomes by the researchers and the variant readings of comparable data. The findings are determined to a large extent by the questions asked, the observations deemed worth making and the kinds of answers that will be intelligible within our existing meaning and value systems. The use of diagnostic categories is a particular problem in such empirical studies, even and especially the category of 'gender identity disorder' itself (Wilson et al., 2002). No psychological study can rise above moral, ethical and ideological considerations. From a postmodern perspective, the methods of psychology can be seen to inappropriately mimic the natural sciences, transforming things 'made' into things 'found' (Shotter, 1993) and thereby concealing the social processes crucial in their production. Such empirical findings can operate 'as truth warrants for a priori commitments to particular forms of value-saturated description' (Gergen, 1992: 24).

Roen (2011a) argues that pioneering gender identity services for children and adolescents, in carefully screening suitable candidates for early puberty suppression, may come to privilege the wishes of those who wish to transition at the expense of those who entertain more uncertainty about their future gender status. Despite the rhetoric about the value of exploration and the toleration of uncertainty, those young people whose gender remains fluid and less assured – their path to gender fixity not realised – risk been seen by services as incoherent or even 'failed' trans subjects. Paradoxically, by expanding the opportunities for trans young people to receive the help they need to lead viable lives, we may also rehearse a new set of normative rules around gender intelligibility. As Roen observes, this move would reflect the tendency in scientific research and writing to 'favour the comparison of clear-cut groups over the examination of complexity' (Roen, 2011b).

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Thinking postmodern and practising postmodern: Implications for practice

Now, the clinician operating from a poststructuralist perspective may not be mightily taken with any of these apparent sources of confidence as to whether the decision to start transitioning for a child or young person is a wise or an unwise one. None seems to constitute a reliable source of knowledge; each is available for deconstructive work that serves to undermine it. But if our focus is on the constructed, contingent, unstable and heterogeneous character of subjectivity, of social relations, of gender and of knowledge, what can we offer our gender dysphoric and determined young clients? Are we not bound to interpret their requests for hormonal and surgical sex reassignment as demanding too much coherence and clarity? Should we not be trying to help them to celebrate fragmentation, and disdain their search for integration through physical intervention? How can we go on as clinicians with no foundational beliefs in which to take refuge? Our work requires us to take a working stance towards the clinical situation when young trans client make demands for physical intervention. As a clinical psychologist sympathetic to the impulse toward postmodern theorising, I want to sketch some of my thoughts in response to this challenge.

If the idea of living in the postmodern era means anything, it is that in all our activity together we are in the business of making meaning in the absence of traditional assurances. In our time, it is hard to see any knowledge or understanding as 'mirroring' nature, or 'mirroring' reality. The idea that most of what have been considered to be the 'natural' facts of human life are better understood as social constructions means that we can no longer picture ourselves simply as observers, critics, knowers – but rather as reflexive understanding actors, as *doers*. Alone with our thoughts, in our smallest gatherings, or operating at the level of a wider community, we are either playing a part in re-inscribing hand-me-down meanings developed through human conversations and shared activities, or we are contributing to the invention of altered meanings and understandings. Butler emphasises (2004) the paradox inherent in our human agency: the self is constituted by social norms and dependent on them, but also 'endeavours to live in ways that maintain a critical and even transformative relation to them'. One does not detect or determine one's gender in a social and relational vacuum. 'One only determines *one's own* sense of gender', Butler continues (tellingly italicising '*one's own*'), 'to the extent that social norms exist that support and enable that act of claiming gender for oneself' (Butler, 2004: 7).

There is an implication here for our work in gender identity clinics: that we are in the business of helping actively to construct the idea and the understanding of transgender, and for this we should accept responsibility. We can see our narrow clinical role with gender variant people – assessing, diagnosing, classifying, referring for specialist treatment and so on – as constructing and enforcing the normalising classification, as if the term 'adolescent with gender disphoria' referred to a definite coherent kind of entity.[3] We can also see our broader work in clinics – listening to our young clients' elaborations of the complexity of their

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gendered experience, being curious about how they do their gender, opening up for discussion the impact of their atypical gender on their family and other relationships – as not just reflecting but contributing to shifting societal understandings of the possibilities of gender and gender identifications. The 'human kinds' (Hacking, 1999) we help to realise and define in gender identity clinics are products of legislation, medical research, psychiatric practice, activism, newspaper articles, television 'soaps' and, crucially, the activism of trans people themselves. The content of our classification of 'kinds', which shifts over time, will define thresholds of what is to count as trans and its variants; it will set norms of typical behaviour, attitudes and aspirations; it may include a causal account; it will offer a range of identities.

And the classification, which is largely a contingent matter, and the social matrix of ideas and practices within which it is embedded, can change how some transgender people think and feel about themselves, their experiences and their actions. Concepts, practices and people overlap, and the classification, in its complex social matrix, are re-constructed. Indeed, it is clear in everyday and clinical settings that different ways of being trans are increasingly on display. What is at stake in working with trans people in a state-funded service is less the specification of the trans predicament as a diagnosable condition, but an elaboration of the *social identity* of the transgender person – how she or he is 'positioned' in relation to others, what that position permits him or her to be, and what self-narratives s/he may fashion. This 'positioning' (Davies and Harre, 1990) has to do with the rights and duties, the privileges and obligations, afforded to him or her in the context of their social world.

Sometimes clinicians do this work of constructing (and deconstructing) transgender in the intimacy of the therapy room, at other times we do the work alongside practitioners from other disciplines that have developed ways of investigating the trans predicament – endocrinologists, developmental scientists, sociologists, psychoanalysts and so on. The postmodern turn has raised epistemological problems for these disciplines, as we have seen, and so we have a commitment to scrutinise their inevitable socio-political underpinnings. But this does not negate in principle the usefulness of their findings in certain contexts of speaking and acting. They represent the traditions on which deconstruction can build its novel meanings. Postmodernism is not a break with the past, however critical it may be of traditional or mainstream ways of understanding, and postmodern therapists should have a good multi-disciplinary grasp of the ways in which these different disciplines approach questions of gender. Nor is it easy to discern once and for all which of these different understandings are compatible with each other, and which contradictory: depending on the context of argumentation, they can be construed as either. We cannot aspire to a grand final mapping or overview of the inputs from these different forms of knowledge.

If, as a clinician, I feel uncomfortable about aspects of how transgender identities are diagnosed psychiatrically, defined politically, parodied culturally or treated surgically – I have a part to play in voicing my concerns in the various settings I inhabit, from the therapy room to the House of Commons to the clinical team meeting to the pub. I may not have a foundational grounding for my beliefs or

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misgivings – they are my best well-informed thoughts – but I do accept my role as actively helping to configure transgender in our time. As such, working clinically in this field is acutely a matter of taking and promoting an ethical and accountable stance. If postmodernism provides us with an epistemic relativity in which all beliefs are seen as socially produced, this does not mean that all beliefs must be held to be equally valid; rather, 'in the forum of scientific judgement, questions of justice take an equal place with those of truth' (Kvale, 1992).

The lack of a foundational theory about how gender and transgender works does not mean that 'anything goes'. Quite the opposite; I have strong views on the way that culture and power thread through all our thinking and the thinking of others, which leads me to introduce certain ideas into therapeutic work with considerable passion:

- I promote a grasp of the possibilities for varied trans identities in order to challenge a passive relationship to diagnosis and classification;
- I create spaces in the work to encourage thinking critically about conventional notions of masculinity and femininity;
- I question the attraction of fixed categories of meaning, recognising that part of the work of therapy is to help clients to try and evolve new meaning, where the existing categories are limiting and confusing;
- I try to be alert to the dangers of a professional preoccupation with causal and developmental stories, while acknowledging the value of a good narrative to a stable sense of self and an intelligible public presence;
- If I see knowledge about the origins of gender identity as radically uncertain, and best theorised as a contingent, mutable social construction, this does not imply that I will see any individual's experience of gender as tentative and unstable;
- I note the struggle for effective communication and adequate self-storying when the linguistic resources for doing justice to the trans experience are so impoverished;
- I am, like Harris (2004) 'always mediating multiple levels of awareness', trying to stay attuned to the way that consciousness of the body emerges from the relational, cultural and social context in which the individual lives;
- I think about the contexts in which endocrinological and other clinical services may present medical interventions as a way to sidestep complex psychological dilemmas;
- I acknowledge that while the technology of altering the body is desired by some to secure a coherent personhood, for some it may make too many demands for conformity within the binary gender divide;
- I especially recognise the comforts of certainty and coherence when I see young people apparently suffering from the fragmenting effects of disrupted and unreliable attachments;
- I give up wondering *why* people's gender identity evolves atypically, and ask instead *how* they are living their gender (Corbett, 1996), and what support they

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need to do this well, where the issue of how to live 'well' is central to the therapeutic work.

The work of clinicians should not be deemed a matter of coming up with an increasingly 'accurate' description of variant gender identity and then working out how best to manage it – whether to pathologise it, normalise it, treat it medically or whatever. It is more the case that clinicians should be accountable in a process of finding and inventing new ways to think about gender and transgender, and to act in relation to transgender people, in a recursive circle. And then – far from succumbing to a paralysing relativism – our task is to question our complex investments in the positions we adopt (Fairfield, 2002). The thinking we do as clinicians – absorbing and filtering theory, polemic and empirical findings about cross-gender identifications – is not a matter of computing and calculating; it is essentially an argumentative process (Billig, 1986), involving the creation, challenge and interpretation of old and novel meanings.

Meaning about transgender is made in families, therapy rooms, schools, peer groups, at medical committees and conferences. We should promote curiosity about the ideas of others within and amongst these groups. We should listen to the contrasting ways of understanding the trans experience and strive to bring them together in new ways. At different times different groups dominate the discourse, while other voices fight to be heard but at a later time perhaps seize the agenda for a while. In the therapy room, therapist and client draw on these different vocabularies and matrices of meaning, and our task is always to resist speaking one dimensionally; this is the reflexive work of therapy. The decision to recommend physical treatment for young people is then a genuinely shared but imperfect decision, involving the client, family, other professionals in the context of a wider cultural world, in which the meaning of trans is constantly shaped and re-shaped, but which rests on no foundation of truth. The therapist is not burdened with needing to be right or certain, but to offer a reflexive and thoughtful space to help clients explore the architecture and borders of their gendered world view.

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Notes

1. Currently, a handful of countries permit the prescription to children of 12 years or older of puberty-blocking drugs, whose effects are designed to wear off should the patient discontinue use; however, the long-term effects of chemically delaying puberty are virtually unknown.
2. For Baumann, postmodernity is

> no more (but no less either) than the modern mind taking a long, attentive and sober look at itself, at its condition and its past works, not fully liking what it sees and sensing the urge to change. Postmodernity is modernity . . . discovering the

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intentions it never before spelled out, finding them mutually cancelling and incongruous (Baumann, 1991: 272)

3. The diagnostic classification of 'Gender Identity Disorder' was replaced in 2013 with 'Gender Dysphoria' in DSM 5 (APA 2013).

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Author Biography

Bernadette Wren trained as a Clinical Psychologist and Systemic Psychotherapist and is now trust-wide Head of Psychology at the Tavistock and Portman NHS Trust. She has degrees in philosophy and psychology and a continuing interest in the relevance of each discipline to the other. She works clinically with gender variant young people and their families in the Trust's Gender identity Development Service. She is an Honorary Senior Research Associate in the Department of Clinical, Educational and Health Psychology at the University College London.

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